

Standard Operating Procedure

Initial Assessment and Care Plan Documentation

«Hospital Name» — Supports AAC.4.a through AAC.4.g

SOP No.	[Hospital to define]	Version	1.0
Effective Date	_____	Review Frequency	Annual, or on change
Prepared By	Quality Coordinator Name: _____ Signature: _____		
Approved By	Medical Superintendent Name: _____ Signature: _____		

1. Purpose

This SOP describes how «Hospital Name» performs standardised initial assessment for every patient, within a defined time, by qualified personnel — resulting in a documented, countersigned care plan for in-patients that identifies special discharge needs early.

2. Scope

Applies to treating doctors, nurses, and other caregivers performing initial assessment in OPD, day-care (including dialysis), in-patient wards, and emergency.

3. Responsibility

Treating Doctor / Assessor: performs assessment within their privileged scope and within 24 hours of ward arrival for in-patients.

Nursing Staff: complete the nursing assessment at admission for IP and day-care.

Clinician In-Charge: countersigns every care plan within 24 hours.

Department Heads: maintain the assessment privilege list; the Medical Superintendent approves it.

4. Procedure

4.1 Standardised initial assessment

1. Use the approved standardised format for the area (OPD, day-care, IP, or emergency). Formats may differ by department but stay consistent within a given area.
2. In emergency, record vital parameters immediately as part of initial assessment.
3. For in-patients, complete history, examination including vitals, drug allergies, provisional diagnosis, and medication reconciliation. If a detailed assessment was already done the same day in OPD or emergency, do not repeat it — reference it instead.
4. Use abridged documentation for day-care patients, including dialysis, where appropriate to the setting.

4.2 Assessment only within privileged scope

1. Before performing any assessment, confirm you are named on the current Assessment Privilege List for that type of assessment.
2. Department Heads keep the privilege list current; the Medical Superintendent approves every addition or change.

4.3 Meeting the time frame

1. For in-patients: the clock starts on ward arrival and stops at assessment completion — the maximum is 24 hours, with no exception. Assess unstable or high-dependency patients immediately, or within a shorter time the hospital defines for that category. [Hospital to define: state the shorter time for unstable/high-dependency patients here.]
2. For emergency: record the primary survey and vital signs immediately on arrival, and begin triage immediately, following the World Health Organization's Emergency Triage Assessment and Treatment (ETAT) framework — see Section 8, Reference 1. The hospital's overall time-to-completion target for emergency initial assessment is [Hospital to define].
3. For day-care: complete assessment before the procedure or session begins.

4.4 Nursing assessment at admission

1. Complete a nursing assessment at the time of admission for every IP and day-care patient, using the checklist or template for that speciality or admission type.
2. Identify nursing needs and any special needs, and document within the time frame the hospital has defined for nursing assessment completion. [Hospital to define: state that time frame here.]

4.5 Documented, countersigned care plan

1. For every in-patient, write a care plan in the patient record based on the initial assessment and any available diagnostic results — including provisional/differential diagnosis, relevant investigations, initial treatment, and specific instructions.
2. A junior doctor may initiate the care plan, but the clinician in-charge must countersign it, dated and timed, within 24 hours.
3. If the clinician in-charge will be unavailable beyond 24 hours, the named covering consultant countersigns instead, and the Medical Superintendent is informed.

4.6 Identifying special post-discharge needs early

1. Begin identifying special discharge needs during the initial assessment — not only on the day of discharge. Both nursing and medical assessment contribute.
2. Pay particular attention to extremes of age, restricted mobility, and continuing nursing or rehabilitation needs.
3. Hand off identified special needs to discharge planning as soon as they are identified, following the hospital's discharge process.

5. Records

Supported by all 23 records in the AAC.4 Required Records pack, covering assessment formats, privilege, timing, nursing assessment, care plan content, countersignature, and special-needs identification.

6. Related Documents

- HCO_AAC_4_v2_REWRITE_DRAFT — AAC.4 policy
- SOP — Patient Re-assessment and Care Plan Modification (AAC.5)

7. Review

Reviewed annually, or sooner when assessment formats or the privilege list change.

8. References

- World Health Organization. (2005). Emergency Triage Assessment and Treatment (ETAT) course.
<https://www.who.int/publications/i/item/9241546875>